sarvodaya HEALTHCARE

SARVODAYA HOSPITA

SECTOR-8, FARIDABAD | HELPLINE: 1800 313 14

DISCHARGE ON REQUEST

MRS. ANITA DEVI **MRN : SR1058**

DOB / AGE	: 51 YR / FEMALE
ADDRESS	: VPO-DAYALPUR, FARIDABAD, HARYANA, INDIA, 121002
MOBILE NO	: 9891712099 **REG. NO :** IP359673
PATIENT CATEGORY	: MDINDIA HEALTH INSURANCE TPA PRIVATE LIMITED
WARD INFO	: 2124/SINGLE ROOM/2F-SAVITRI BLOCK
NEXT OF KIN	: MR.SAGAR (SON) PH: 8826989227
CASE SUMMARY	: DISCHARGE ON REQUEST
DEPARTMENT	: INTERNAL MEDICINE
CONSULTANTS	: DR. SUMIT AGGARWAL [INTERNAL MEDICINE]
ADMISSION DATE	: 10-02-2026 **03:12 PM** **DISCHARGE DATE :** 13-02-2026

ALLERGIES : NO KNOWN ALLERGIES, NO KNOWN DRUG ALLERGIES

DOCTOR TRANSFER DETAILS

DATE	DEPARTMENT	CONSULTANT
10-02-2026 03:12 PM	INTERNAL MEDICINE	Dr. SUMIT AGGARWAL

FINAL DIAGNOSIS

DIAGNOSIS	ICD CODE
ACUTE SEVERE GASTRITIS WITH RECURRENT VOMITING	
CHOLELITHIASIS	
ACUTE UTI	
ACUTE VASCULAR HEADACHE	

PRESENTING COMPLAINTS

Patient was admitted with complaints of multiple episode of vomiting 10-12 episodes since today, associated with pain abdomen- epigastric region

EXAMINATION

Physical Findings & Systemic Examinations:

Afebrile

Pulse Rate: 70/ minute

Blood Pressure: 140/90 mm of Hg

Respiratory Rate: 22 / minute

SPO2: 98% on room air

No Pallor/ Icterus/ Pedal Edema/ Cyanosis/ or Lymphadenopathy

Abdomen: Soft, bowel sounds are present

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Respiratory System: Bilateral air entry present, Diffuse tenderness++
CVS: S1S2 audible, no murmur
CNS: No focal deficit

COURSE DURING HOSPITAL STAY

Patient was admitted with above mentioned history and complaints. After clinical examination , provisional diagnosis of acute severe gastritis was made.

Initially patient was managed with IV antibiotics, IV PPI, IV analgesics, IV antipyretics with other supportive treatment and symptomatic measures. Relevant investigation were done which showed
CBC- Hb- 12.8, TLC- 8.72, PLT- 129k.
RFT- Urea- 37.5, S.Cr- 0.62, Na+- 137.3, K+- 4.60.
LFT- T/D/I- Bili- 0.37/0.12/0.25, SGPT/SGOT- 14.5/38.2, Total Protein- 7.00, Albumin- 4.17,
X-Ray Abdomen Errect Position- Fecal and gaseous loading of bowel loops.
X-Ray Chest PA- *No significant abnormality. Lipase- 32.9amylase- 64.3
urine R and m - pus cells- 10-15
On 11/02/2026: patient had complaints of - Pain abdomen -Poor oral intake -Generalized weaknes Nausea. CONTINUE SAME TREATMENT
General surgery reference in view of cholelithiasis and advised followed
Urine Culture and sensitivity: REPORT AWAITED

Ultrasound Whole Abdomen was done in view of abdominal discomfort which showed Cholelithiasis

On 12/02/2026: Patient had complaints of , Pain abdomen reduce , Headache reduce ,Poor oral intake ,Generalized weakness ,Nausea. continue ongoing treatment
On 13/02/2026
Patient require further stay fot stabilisation but attendant requsted for discharge. Hence patient is discharged on request. .

DISCHARGE MEDICATION

SR.	MEDICINE	DOSAGE INSTRUCTION	DUR. / RO
1.	**CAP RMZOL DSR** *DOMPERIDONE 30MG,RABEPRAZOLE 20MG CAP*	1 CAP once daily before breakfast	5 d
2.	**TAB OFLOX 200 TAB** *OFLOXACIN* 200 MG*	1 TAB Two times a day	5 d
3.	**NOS INDERAL LA 20** *PROPRANOLOL 20 MG (20MG)*	1 TAB Two times a day	5 d
4.	**TAB TRYPTOMER 10 MG** *AMITRIPTYLINE 10 MG TAB*	1 TAB at night AFTER DINNER-9pm	5 d

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Age /Gender: 51
Contact No.: 989171
Bill Date: 13-0
Dr. SUMIT AG
te: 10-02-20
dia Health

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5.	**TAB DROTIN** *DROTAVERINE 40 MG*	1 TAB for PAIN ABDOMEN	5 d.
6.	**SYRUP LAXPURE 200ML SYP** *LACTITOL*	20 ml Two times a day	5 d. OR

FOLLOW UP

Do not stop any medication without doctor advice

Consult Doctor/Hospital In Case Of
High grade fever / Vomiting / Pain Abdomen / Giddiness / Palpitation

DIET
Diet as advised
Avoid spicy, oily, fatty meals
No tea, coffee on empty stomach

Condition At Discharge
Conscious/ Oriented
BP-110/70 mmHg
HR-78/min
Afebrile
Spo2-98%
Symptomatically better
Accepting oral feeds well

Follow Up
Follow up TUESDAY with Dr Sumit Aggrawal in internal medicine OPD

Follow up in Gen. Surgery OPD (Dr. Anushtup de)

Contact with Mr. Raju for appointment in OPD (7303111095).

Consult

Dr. SUMIT AGGARW
MBBS, MD (Internal Medici
OPD Timings :- 12:00 PM to 3:00
Room No.:- 1

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